Pharmacovigilance Safety Analysis Report

Adverse Drug Reaction Identification, ICSR Documentation, Causality Assessment & Case Classification

Document ID:	PV-REPORT-TASK1-001	**Date of Report:**	October 09, 2026
Suspect Drug:	Amoxicillin 500 mg Capsule	**Report Type:**	Initial Case Safety Report

1. Task 1: ADR Identification Basics

1.1 Clinical Case Narrative

The patient is a 45-year-old female (P.S., ~62 kg) with a 2-year clinical history of Essential Hypertension well-controlled on Amlodipine 5 mg once daily without prior adverse events. On Day 1, she was prescribed Amoxicillin 500 mg TID for an acute bacterial upper respiratory tract infection (URTI). On Day 4 of antibacterial therapy, she presented with acute widespread erythematous maculopapular rash, severe generalized pruritus, facial flushing, and mild dizziness.

The rapid temporal onset following Amoxicillin initiation and subsequent resolution upon drug withdrawal strongly implicates Amoxicillin as the primary suspect drug, while Amlodipine remains a secondary concomitant agent with long-standing tolerance.

1.2 Patient Baseline & Symptom Timeline Mapping

Timeline Phase	Medication & Dose	Clinical Presentation & Symptoms	Event Classification
Day -730 to Day 1	Amlodipine 5 mg QD	Normal baseline status, blood pressure well-controlled.	Chronic Baseline
Day 1	Amoxicillin 500 mg TID	Cough, sore throat, fever (URTI presentation).	Primary Indication
Day 4	Amoxicillin 500 mg TID	Acute maculopapular rash, severe pruritus, facial flushing, mild dizziness.	Adverse Drug Reaction
Day 5	Amoxicillin Discontinued	Drug withdrawn (Dechallenge); oral H1-antihistamine initiated.	Intervention / Action Taken
Day 6 – Day 7	Drug Withdrawn	Gradual abatement and complete recovery of rash and itching.	Resolution Phase

2. Task 2: Individual Case Safety Report (ICSR)

Section A: Patient Information

Patient Initials	P.S.	**Age / Gender**	45 Years / Female

Weight	62 kg	**Relevant Medical History**	Essential Hypertension (2 yrs), Acute URTI

Section B: Suspected Adverse Reaction

Adverse Reaction Description	Acute widespread erythematous maculopapular rash, severe generalized pruritus, facial flushing, mild dizziness
Date of Reaction Onset	Day 4 post-Amoxicillin initiation
Outcome of Reaction	Recovering / Resolving (following drug dechallenge)

Section C: Suspected & Concomitant Medicinal Products

Drug Role	Drug Name	Dosage & Form	Route	Frequency	Therapy Dates	Action Taken
Suspect	Amoxicillin	500 mg Capsule	Oral	TID	Day 1 – Day 5	Drug Withdrawn
Concomitant	Amlodipine	5 mg Tablet	Oral	QD	Day -730 – Ongoing	Dose Maintained

Section D: Reporter Details & Diagnostic Data

Reporter Type	Pharmacovigilance Specialist / Healthcare Professional
Report Type	Initial Spontaneous Case Safety Report
Diagnostic / Test Data	Clinical observation of erythematous cutaneous rash; symptom resolution post-withdrawal

3. Task 3: Causality Assessment

3.1 Naranjo Algorithm Scoring Matrix

Assessment Parameter / Question	Option	Score	Clinical Rationale
1. Are there previous conclusive reports on this reaction?	Yes	+1	Amoxicillin hypersensitivity and rash are well-documented.
2. Did the adverse event appear after the suspected drug was given?	Yes	+2	Symptoms appeared on Day 4 following initiation on Day 1.
3. Did the reaction improve when the drug was discontinued?	Yes	+1	Rash and pruritus resolved after stopping Amoxicillin.

Assessment Parameter / Question	Option	Score	Clinical Rationale
4. Did the adverse event reappear when drug was readministered?	Not Done	0	Rechallenge not performed for safety/ ethical reasons.
5. Are there alternative causes that could have caused the reaction?	No	+1	Patient has no allergy history; Amlodipine tolerated for 2 yrs.
6. Did the reaction appear when a placebo was given?	No	0	No placebo administered.
7. Was the drug detected in blood/fluids in toxic concentrations?	Unknown	0	Drug levels not measured.
8. Was reaction altered by dose increase or decrease?	Unknown	0	Drug stopped completely rather than dose adjusted.
9. Did the patient have a similar reaction to similar drugs?	Unknown	0	No prior penicillin exposure history recorded.
10. Was the adverse event confirmed by objective evidence?	Yes	+1	Visual observation of erythematous maculopapular rash.
Total Cumulative Naranjo Score		**+6 (PROBABLE)**	

Causality Final Classification

Naranjo Score: +6 (Probable) | **WHO-UMC Category: Probable / Likely**
Dechallenge Assessment: Positive (symptom resolution following drug discontinuation).
Rechallenge Assessment: Not Performed / Ethically Contraindicated.

4. Task 4: Severity & Case Classification

Safety Parameter	Classification	Regulatory & Clinical Rationale
Seriousness Criteria	**Non-Serious**	Does not meet regulatory criteria for death, life-threatening risk, hospitalization, or persistent disability.
Severity Grade	**Moderate (Grade 2)**	Moderate cutaneous distress requiring drug withdrawal and symptomatic antihistamine treatment.
Expectedness	**Expected**	Cutaneous hypersensitivity and rash are explicitly listed in the official package insert for Amoxicillin.
Reporting Timeline	**30 Calendar Days**	

Safety Parameter	Classification	Regulatory & Clinical Rationale
		Standard reporting timeline for non-serious expected spontaneous adverse drug reaction reports.

4.1 Clinical Impact & Regulatory Action Plan

1. **Medical Record Alert:** Document a permanent *Penicillin / Beta-lactam Allergy Alert* in the patient's Electronic Health Record (EHR).
2. **Alternative Antimicrobial Selection:** Transition to a non-beta-lactam agent (e.g., Azithromycin) if antibiotic therapy for the upper respiratory tract infection remains clinically required.
3. **Symptomatic Treatment:** Administer oral H1-antihistamines (Cetirizine 10 mg QD) until complete resolution of cutaneous symptoms.
4. **Pharmacovigilance Filing:** Submit this ICSR to the internal safety database and transmit the standard report to the national pharmacovigilance regulatory body.